Lenvatinib: differentiated thyroid cancer

National Cancer Control Programme (NCCP) 00295 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before starting

Full blood count, kidney and liver tests, coagulation, urine protein, thyroid-stimulating hormone, electrocardiogram (ECG) and blood pressure. Urine protein must be below 1 g/24 hours at initiation; blood pressure must be controlled.

STEP 2

Taking or receiving treatment

Take at about the same time each day, with or without food. Swallow capsules whole with water; do not crush or break them. A pharmacist can teach suspension preparation: place up to 5 whole capsules in a small container or 20 mL oral syringe, add 3 mL water, apple juice or milk, wait 10 minutes, then stir or shake for 3 minutes. Take all the mixture, add another 2 mL liquid and take the rinse. For a feeding tube use water only. If a missed dose cannot be taken within 12 hours, skip it and take the next dose at the usual time.

STEP 3

Tests and reviews

Every 28 days: blood count, kidney tests, coagulation, urine protein and thyroid-stimulating hormone. Liver tests every 2 weeks for the first 2 months, then monthly; calcium monthly. Blood pressure after week 1, every 2 weeks in the first 2 months, then monthly. ECG when clinically indicated.

STEP 4

Specific practical instructions

Treat diarrhoea and vomiting promptly using the agreed plan because dehydration can damage the kidneys. Check blood pressure as taught. Report sudden severe headache, vision changes, bleeding, chest pain, severe abdominal pain or fluid leakage from an abnormal opening urgently.

STEP 5

Leaving with a clear plan

Confirm the next appointment, dose, support medicines and verified daytime and out-of-hours phone numbers.

Sources: NCCP 00295 v6: official regimen. Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending.